

SENATE CHAMBER

STATE OF OKLAHOMA

DISPOSITION

☒ FLOOR AMENDMENT

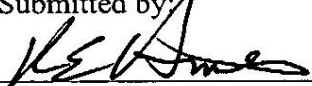
No. 1

☐ COMMITTEE AMENDMENT

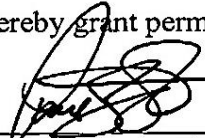
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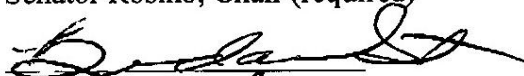
I move to amend House Bill No. 3644 by substituting the attached floor substitute (Request #3984) for the title, enacting clause and entire body of the measure.

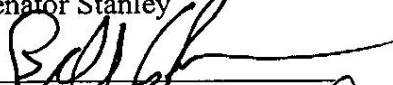
Submitted by:

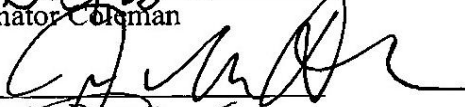

Senator Hines

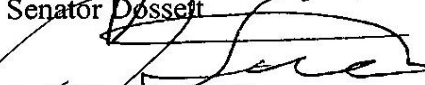
I hereby grant permission for the floor substitute to be adopted.

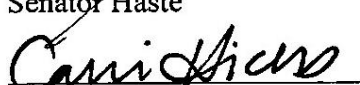

Senator Rosino, Chair (required)


Senator Stanley


Senator Coleman

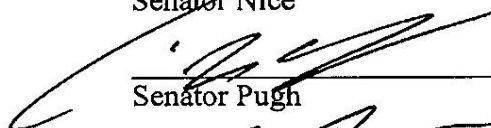

Senator Dossett

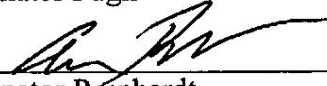

Senator Haste


Senator Hicks

Senator McIntosh

Senator Nice


Senator Pugh


Senator Reinhardt

Senator Standridge

Senator Thompson

Senator Paxton, President Pro Tempore

Senator Daniels, Majority Floor Leader

Note: Health and Human Services Committee majority requires seven (7) members' signatures.

Hines-DC-FS-HB3644

4/23/2026 1:34 PM

(Floor Amendments Only)

Date and Time Filed: 4.27.26 2:39pm jd

☐ Untimely

☐ Amendment Cycle Extended

☐ Secondary Amendment

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

FLOOR SUBSTITUTE

FOR ENGROSSED

HOUSE BILL NO. 3644

By: Stinson and Archer of the
House

and

Hines and Mann of the
Senate

FLOOR SUBSTITUTE

[venous thromboembolism - hospitals - training -
State Department of Health - registry - requirements
- reports - assisted living centers - definitions -
rules - information - noncodification - codification
- effective date -
emergency]

~~BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:~~

SECTION 1. NEW LAW A new section of law not to be
codified in the Oklahoma Statutes reads as follows:

This act shall be known and may be cited as the "Blake Burgess
Act".

SECTION 2. NEW LAW A new section of law to be codified
in the Oklahoma Statutes as Section 1-630 of Title 63, unless there
is created a duplication in numbering, reads as follows:

1 A. Each hospital with an emergency department and each
2 ambulatory surgical center shall:

3 1. Develop and implement policies and procedures for the
4 rendering of appropriate medical attention for persons at risk of
5 forming venous thromboembolisms which reflect evidence-based best
6 practices relating to, at a minimum:

7 a. assessing patients for risk of venous thromboembolism
8 using a nationally recognized risk assessment tool,
9 and

10 b. treatment options for a patient diagnosed with venous
11 thromboembolism; and

12 2. Train all nonphysician personnel at least annually on the
13 policies and procedures developed under this section. For purposes
14 of this section, the term "nonphysician personnel" means all
15 personnel of the licensed facility working in clinical areas and
16 providing patient care, except those persons licensed as health care
17 practitioners.

18 B. Nothing in this section shall be construed to allow or
19 require personnel to practice outside the scope of their
20 certification, license, or qualification.

21 SECTION 3. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 1-631 of Title 63, unless there
23 is created a duplication in numbering, reads as follows:

1 A. The State Department of Health shall contract with the
2 state-designated entity for health information exchange to establish
3 and maintain a statewide venous thromboembolism registry to ensure
4 that the performance measures required to be submitted under
5 subsection B of this section are maintained and available for use to
6 improve or modify the venous thromboembolism care system, ensure
7 compliance with nationally recognized guidelines, and monitor venous
8 thromboembolism patient outcomes.

9 B. Beginning July 1, 2027, each hospital with an emergency
10 department shall electronically report annually to the statewide
11 venous thromboembolism registry information containing nationally
12 recognized venous thromboembolism measures and data on the incidence
13 and prevalence of venous thromboembolisms. Such data shall include
14 the following information:

15 1. The number of venous thromboembolisms identified and
16 diagnosed;

17 2. The age of the patient;

18 3. The ZIP code of the patient;

19 4. The sex of the patient;

20 5. Whether the patient is a resident of a licensed nursing
21 facility or assisted living center;

22 6. Whether the venous thromboembolism was fatal;

23 7. How the diagnosis was made, such as by using imaging
24 modalities; and

1 8. The treatment that was recommended for the venous
2 thromboembolism.

3 C. The Department shall require the state-designated entity for
4 health information exchange to collect data from each hospital with
5 an emergency department on the performance measures required under
6 subsection B of this section. The state-designated entity shall
7 provide to the Department regular reports on the data collected.

8 D. By June 1, 2027, the Department shall electronically submit
9 to the Governor, the President Pro Tempore of the Senate, and the
10 Speaker of the House of Representatives a detailed report on the
11 incidence of venous thromboembolism using inpatient and outpatient
12 data for services provided between the effective date of this act
13 and June 30, 2027. The report shall provide analyses of all of the
14 following:

15 1. Age category, initial primary diagnosis and procedure, and
16 secondary diagnoses, readmission rates for inpatients, admission
17 rates for venous thromboembolism for which the patient had an
18 ambulatory surgery procedure, and emergency department visits for
19 venous thromboembolism linked to any previous admission;

20 2. Whether the venous thromboembolism was present upon
21 admission;

22 3. The incidence of venous thromboembolism procedures reported
23 on the website of the Department; and
24

1 4. The principal payor, the sex of the patient, and the
2 patient's discharge status.

3 E. The state-designated entity for health information exchange
4 operating the registry shall only use or publish information from
5 the registry for the purposes of advancing medical research or
6 medical education in the interest of reducing morbidity or
7 mortality.

8 SECTION 4. AMENDATORY 63 O.S. 2021, Section 1-890.2, is
9 amended to read as follows:

10 Section 1-890.2. As used in the Continuum of Care and Assisted
11 Living Act:

12 1. "Assisted living center" means any home or establishment
13 offering, coordinating or providing services to two or more persons
14 who:

- 15 a. are domiciled therein,
- 16 b. are unrelated to the operator,
- 17 c. by choice or functional impairments, need assistance
18 with personal care or nursing supervision,
- 19 d. may need intermittent or unscheduled nursing care,
- 20 e. may need medication assistance, and
- 21 f. may need assistance with transfer and/or ambulation;

22 2. "Board" means the State Board of Health;

23 3. "Commissioner" means the State Commissioner of Health;

1 4. "Continuum of care facility" means a home, establishment or
2 institution providing nursing facility services as defined in
3 Section 1-1902 of this title and one or both of the following:

4 a. assisted living center services as defined in the
5 Continuum of Care and Assisted Living Act, and

6 b. adult day care center services as defined in Section
7 1-872 of this title; ~~and~~

8 5. "Department" means the State Department of Health;

9 6. "Pulmonary embolism" (PE) means a condition in which part of
10 a clot breaks off and travels to the lungs, possibly causing death;
11 and

12 7. "Venous thromboembolism" (VTE) means deep vein thrombosis
13 (DVT), which is a blood clot located in a deep vein, usually in the
14 leg or arm. The term can be used to refer to DVT, pulmonary
15 embolism, or both.

16 SECTION 5. AMENDATORY 63 O.S. 2021, Section 1-890.3, as
17 amended by Section 1, Chapter 357, O.S.L. 2025 (63 O.S. Supp. 2025,
18 Section 1-890.3), is amended to read as follows:

19 Section 1-890.3. A. The State Commissioner of Health shall
20 promulgate rules necessary to implement the provisions of the
21 Continuum of Care and Assisted Living Act. Such rules shall
22 include, but shall not be limited to:

23 1. A uniform comprehensive resident screening instrument to
24 measure the needs and capabilities of residents in all settings and

1 to determine appropriate placements of residents. Such assessment
2 shall include screening for the signs and symptoms of venous
3 thromboembolism (VTE) and techniques for providing an emergency
4 response;

5 2. Physical plant requirements meeting construction and life
6 safety codes, with provisions accommodating resident privacy and
7 independence in assisted living centers and in assisted living
8 components of continuum of care facilities based on the variable
9 capabilities of residents;

10 3. Staffing levels responsive to the variable needs of
11 residents, with provisions for sharing of staff between components
12 in a continuum of care facility;

13 4. Minimum standards for resident care including, but not
14 limited to, standards pertaining to medical care and administration
15 of medications. Standards pertaining to medication administration
16 shall, at a minimum, require the assisted living center or continuum
17 of care facility to:

- 18 a. provide or arrange qualified staff to administer
19 medications based on the needs of residents,
- 20 b. follow medication administration orders from a
21 qualified health care provider,
- 22 c. ensure that medications are reviewed monthly by a
23 Registered Nurse or pharmacist and quarterly by a
24 consultant pharmacist,

d. maintain medication administration records and document all medication administration in such records, and

e. have medication storage and disposal policies;

5. Standards for measuring quality outcomes for residents;

6. Provisions for individualized services chosen by and designed for each resident;

7. Provisions to prohibit facility staff from disclosing a resident's financial information to third parties without written consent of the resident or the designated representative of the resident;

8. Procedures for inspections and investigations of licensed entities to ensure compliance with the Continuum of Care and Assisted Living Act and rules promulgated by the Commissioner;

9. Enumeration of resident rights and responsibilities to be observed by each facility and its staff. Such resident rights shall include the freedom of choice regarding any personal attending physicians and all other providers of medical services and supplies, providing that the minimum standards are met by the provider pursuant to the Continuum of Care and Assisted Living Act, without a financial penalty or fee charged by the assisted living center;

10. Provisions for a surety bond or deposit from each applicant in an amount sufficient to guarantee that obligations to residents will be performed, with provisions for reduction or waiver of the

surety bond or deposit when the assets of the applicant or its contracts with other persons are sufficient to reasonably ensure the performance of its obligations;

11. Provisions requiring assisted living centers to make available to residents upon admission a consumer information pamphlet that contains information about venous thromboembolism (VTE) including, but not limited to, risk factors and recognition of the signs and symptoms of VTE;

12. Provisions for the development of a consumer guide or similar resource to be posted on the Internet website of the State Department of Health to assist individuals and families in understanding the services provided by assisted living centers and to compare and select a facility;

~~12.~~ 13. Provisions for posting results of routine inspections and any complaint investigations of each assisted living center on the Internet website of the Department. Such information shall be regularly updated to include the facility's plan of correction and to indicate when a violation of a licensing regulation was corrected by the facility; and

~~13.~~ 14. Provisions requiring execution of a plan of care and a resident service contract with the resident or resident's representative.

B. The nursing care service of a continuum of care facility shall be subject to the requirements, procedures and remedies set

1 out in the Nursing Home Care Act, including provisions relating to
2 resident rights.

3 C. The adult day care component of a continuum of care facility
4 shall be subject to requirements and procedures specified under the
5 Adult Day Care Act.

6 SECTION 6. This act shall become effective July 1, 2026.

7 SECTION 7. It being immediately necessary for the preservation
8 of the public peace, health or safety, an emergency is hereby
9 declared to exist, by reason whereof this act shall take effect and
10 be in full force from and after its passage and approval.

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12 60-2-3984 DC 4/27/2026 3:49:24 PM

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